



The Global Burden of Mental Illness

Prevalence, Impact, and the Treatment Gap

Global data overview for public health policy analysts and mental health program planners

Evidence source

Our World in Data / IHME Global Burden of Disease (2025)

Authors: Saloni Dattani, Lucas Rodés-Guirao, Hannah Ritchie, and Max Roser (2023)

Hundreds of Millions Affected

Lifetime major depression risk underscores the scale of need for policy response.

Women



1 in 3

will experience major depression
in their lifetime

Men



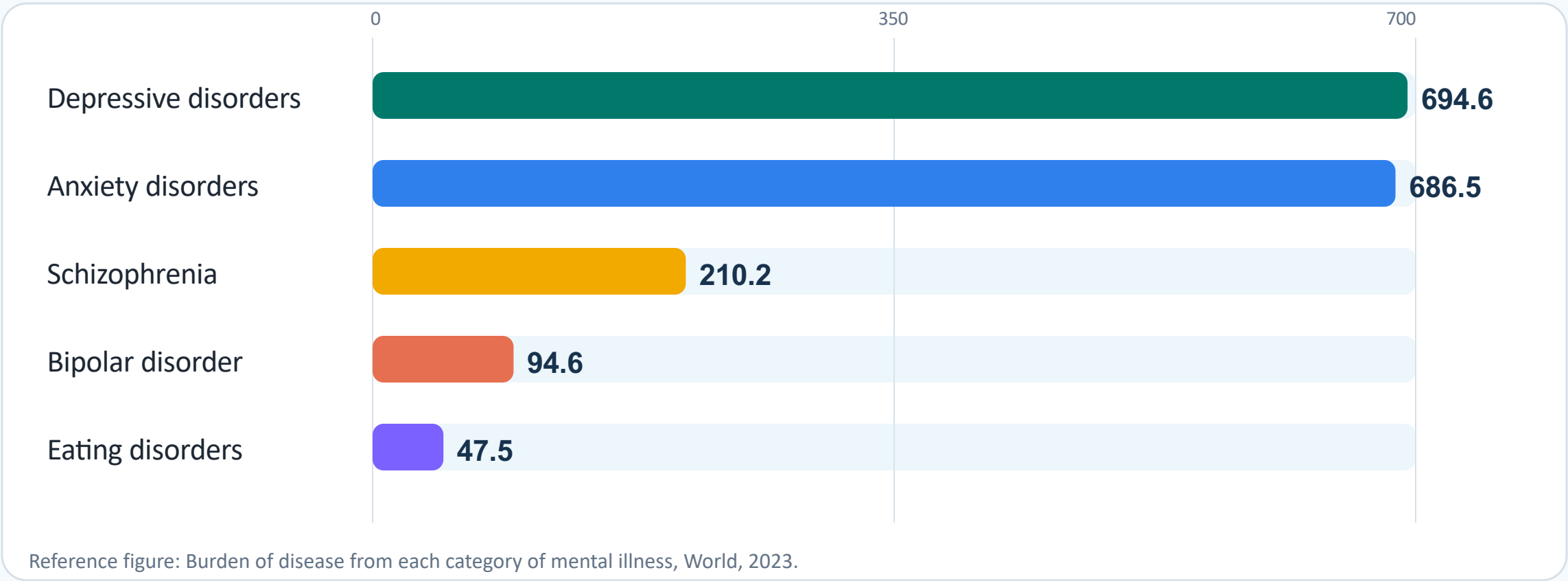
1 in 5

will experience major depression
in their lifetime

Mental health conditions span common disorders such as depression and anxiety, and less common but severe conditions such as schizophrenia

Depression and Anxiety Dominate the Disease Burden

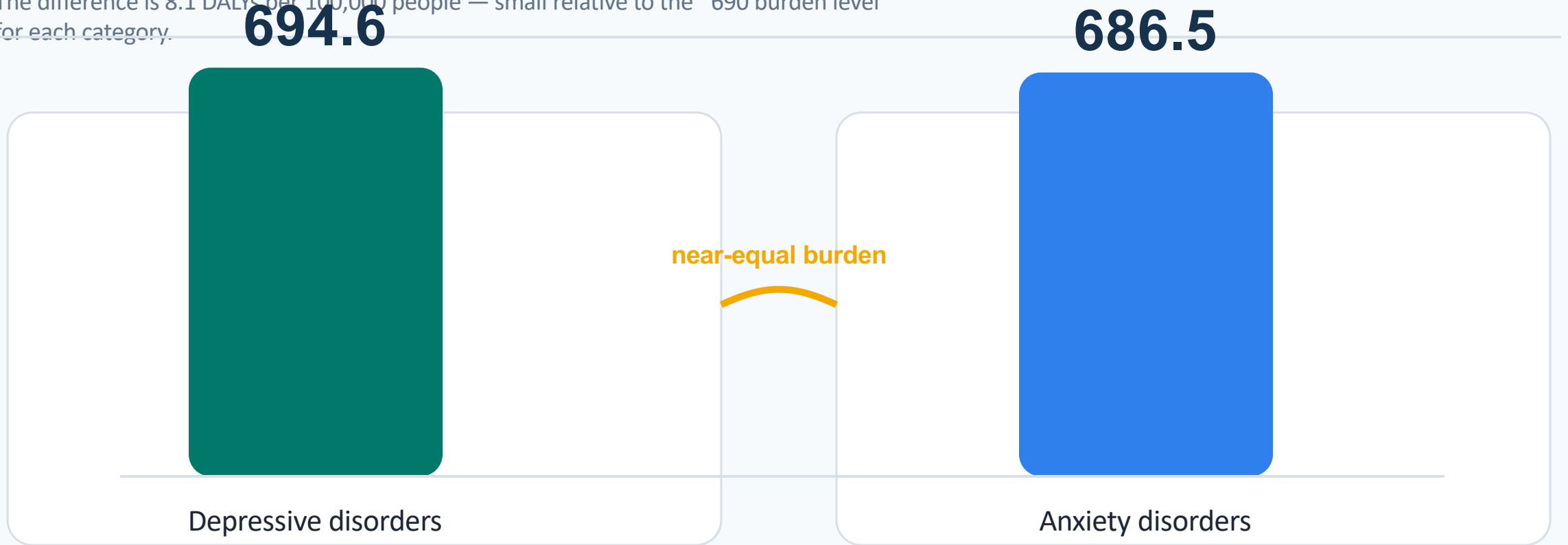
DALYs combine years of life lost due to premature death and years lived with disability.



NEAR PARITY

Depression and Anxiety Carry Nearly Equal Burden Worldwide

The difference is 8.1 DALYs per 100,000 people — small relative to the ~690 burden level for each category.

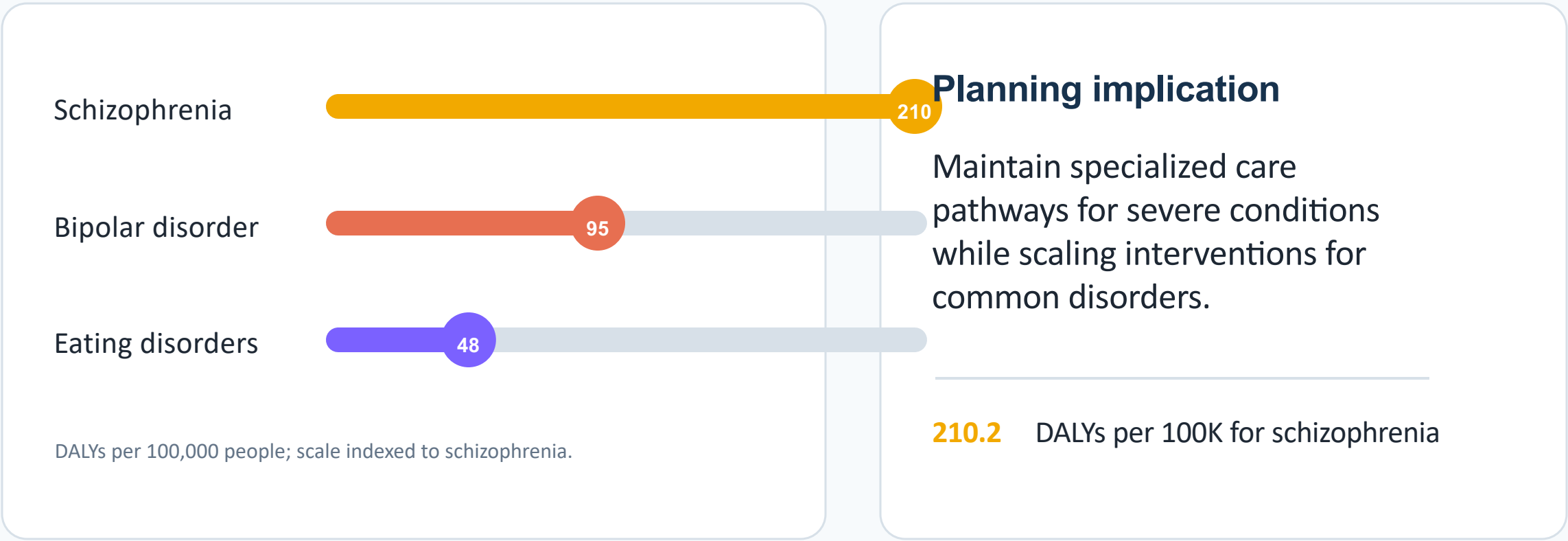


Together, depression and anxiety dominate the global mental health DALY burden across these five categories.

SEVERE CONDITIONS

Schizophrenia and Bipolar Disorder: Lower Population Burden, Severe Individual Impact

Population-level DALYs are lower than depression or anxiety, but the source highlights high individual disability and severe impact.



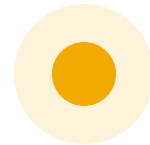
Treatment Is Available — But Often Lacking or Poor Quality

Mental illnesses are treatable, yet stigma, access, quality, and underreporting constrain response.



Stigma

People may feel uncomfortable disclosing symptoms to professionals or people they know.



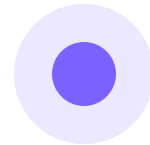
Access

Treatment is often lacking, especially in low- and middle-income countries.



Quality

Even where treatment exists, it may be of poor quality.



Underreporting

Discomfort with disclosure makes true prevalence difficult to estimate.

The actual burden may be higher than reported figures suggest.

How People Cope: Medication, Faith, and Family Conversations

Responses to anxiety or depression vary widely across countries and cultures.



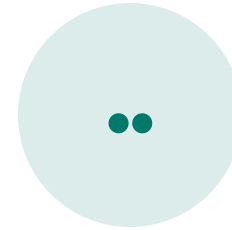
Taking prescribed medication

Common in higher-income settings



Religious or spiritual activities

Prevalent in many regions worldwide



Talking to friends or family

Widely reported coping mechanism

Policy interventions must account for cultural, economic, and healthcare system differences rather than one-size-fits-all approaches.

Key Takeaways for Policy and Program Planning

Scale interventions for common disorders while maintaining care for severe conditions.

1

Prioritize depression and anxiety

They represent the overwhelming majority of mental health DALYs across the five listed categories.

2

Maintain specialized severe-condition care

Schizophrenia and bipolar disorder carry disproportionate individual burden.

3

Close the treatment gap

Treatment exists, but access and quality remain inadequate.

4

Address stigma and data quality

Stigma suppresses help-seeking and accurate prevalence estimates.

5

Design culturally sensitive responses

Coping strategies vary widely across countries and cultures.

